جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساميه حنا عجبانيي عبدالمسيح
المحافظة : الجيزه **الحي / المدينة :** الوراق **العنوان :** ترعه السواحل
رقم البطاقة : 26410142600422 **رقم التليفون :** 01155514493 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 14-10-1964 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساميه حنا عجباني عبدالمسيح
National ID: 26410142600422

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



DMS

مقطعية صدر وبطن وحوض

ماموجرام



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MDT form

A-Patient ID
1- Name : [redacted] Sex : Female
3- Medical number : 03204753
Old Med :
2-ID number : 30410142600422 Patient Age : 60 Y
1358-2024

0 0

Family history	Phone	01155511893	BMI	REASON of MDT
marital status	NRS Score		GROUP	Date of MDT 2025-05-06
Comorbidity	Menopausal status		Initial STAGE	

Clinical data

Breast Imaging :

Mets Workup :
PET CT FREE

BX	ER	PR	Her 2 Neu	SISH	KI67
ILC	8/8	7/8	0		30

T	N	M
1m	3b	0

Sonomammography

PATIENT HISTORY

she received 5months AI
pet ct: morphologic progression of axillary LN
received 5 months AI followed by PD-->4 AC+4 taxotere
LT BCS+ALND: free margin
YPT0 , N1 (1/18)

MDT OUTCOME & PLAN

ADJUVANT FEMARA
PORT
ZOMETA/6MONTHS

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الاسم / منيه حنا عطيتي عبدالمسيح	Request No.	: 13006907
Patient No.	: 26410142600422	Request Date	: 09/05/2024
Path No.	: SR-577/24	Verification Date	: 16/05/2024
Sex	: Female	Financial Category	: وزارة الصحة - علاج الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:
- 6 paraffin blocks labeled by (635690) was received.

Microscopic Examination:
* Histological type: Invasive pleomorphic lobular carcinoma.
* DCIS: No.
* Angioinvasion: No.
* Perineural invasion: No.
* IPT:
- ER: Strong nuclear reaction in 85% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 7/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 30% nuclear reaction of tumor cells.

Diagnosis:
Left breast cancer, Slide revision:
Invasive pleomorphic lobular carcinoma.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 30% of tumor cells

Signature

Verified by :

CAP
غرفة رقم (1) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (1) - First Floor-Building No. 987- Nile Cornicha - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
إدارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Samia Hanna Agaybi A Msih	Date : 18/07/2024
Exam : MAMMOGRAPHY BOTH	Age : 59 Year(s)
Medical ID: 1388/2024	Pat. ID: 76476

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathologically proven malignant mass on neoadjuvant hormonal therapy for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows LOQ deeply seated spiculated irregular mass lesion, of equal density currently measuring about 1.9x1.8 cm , with clip seen within
- No spiculated masses or suspicious microcalcifications on the right breast
- Complementary ultrasound revealed left breast irregular spiculated isoechoic mass lesion, seen at 8 o'clock measuring 2x1.7 cm (was 2x1.8 cm, in prior study dated)
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific right axillary LN.
- Pathological left axillary (level I and II) lymph nodes.
- Free supra-clavicular regions.

Opinion:

- No radiological response of the left breast LIQ malignant mass (BIRADS 6)
- Pathological left axillary lymph nodes.
- Benign findings right breast (BIRADS 2)

Yours truly,

Dr/Asmaa Zoinhom, Msc

Not Approved

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	:	Request No	: 13005120
Patient No.	: 2641014260422	Request Date	: 08/04/2025
Path No	: 1220/25	Verification Date	: 14/04/2025
Sex	: Female	Financial Category	:
Age	: 60 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, post NACT
Nature of specimen: Left BCS &left axilla
Gross examination:
- Procedure: Left BCS & Axillary clearance

Fibrofatty tissue piece measured 5.5x5.5x3 cm with covering skin ellipse measured 5.5x3 cm

- Tumor extent: A greyish white tumour bed site (CLIPPED AWARE)
- Size: 1x1 cm.
- Skin involvement: Unremarkable grossly
- Distance from margins: 0.2 cm away from closest inferior margin

-Extradeep margin: Fibrofatty tissue piece measured 2.5x1 cm, free of gross induration
-Extrainferior margin: Fibrofatty tissue piece measured 2.5x2.5 cm, free of gross induration
-Axilla: Fibrofatty tissue piece measured 6x6 cm containing 18 lymph nodes up to 3x1.5 cm

- Frozen section: Free margins

Microscopic examination
- Histological type: No residual tumor
- Ductal carcinoma in situ: No
- Lymph nodes: Positve (3/18)
- Skin involvement: Free
- Resection margins: Free margins
- Histologic response to therapy: Pathologic partial response

Diagnosis:
Left BCS &Axillary clearance (post NACT)
No residual tumor
- Free margins
- Lymph nodes positive (3/18)
- Pathologic partial response (PPR)
- Stage: ypT0 N1

Verified by:
Dr Heba Hasan

Room No. (B) - 2nd Floor Building No. 35 - Nile Corniche - Abo El Qodous - Cairo - Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

atient Name		Request No	: 13605120
atient No	: 26410142600122	Request Date	: 08/01/2025
ge No	: 1220/25	Verification Date	: 14/01/2025
ex	: Female	Financial Category	:
ge	: 60 Y	Department	: Out Patient
eferring Doc.	:	Registered by	: Mrs Mayada Hegazy

Pathology Report

RCB : 1.516 , RCB class II

Dr. Rana Fathy
Dr Heba Hassan

Verified by
Dr Heba Hassan

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Center
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Samia Hanna Agaybi A. Msih	Date : 06/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 60 Year(s)
Medical ID: 1388/2024	Pat. ID: 76476

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast LIQ metallic structure with no underlying gross lesions.
- Multiple left axillary level I & II enlarged lymph nodes largest measuring 2.1 x 1.8 cm.
- Prominent left internal mammary lymph node measuring 5 X 10 mm.
- Tiny left supraclavicular lymph node measuring 0.5 cm.
- Few bilateral subcentimeteric subpleural pulmonary nodules largest at right upper lobe posterior segment measuring 7 mm having a fissural base.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral thyroid hypodense nodules with heterogeneous texture.
- Bilateral elastofibroma dorsi.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries
- Average size spleen with few hypodense non enhancing focal lesions largest at lower pole measuring 1.5 x 1.2 cm mostly cysts.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Right renal simple cortical cyst.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Samia Hamed Agag El A. Msih Exam : C T CHEST WITH CONTRAST Medical ID: 1388/2024	Date : 08/03/2025 Age : 60 Year(s) Pat ID: 76476

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Element of mesentric panniculitis.

Scanned skeleton: no definite worrisome lytic or sclerotic focal lesions.

- L3 small lytic lesion with vertical striation likely hemangioma.

Opinion:

- ***Left breast UIQ metallic structure with no underlying gross lesions.***
- ***Multiple left axillary pathological lymph nodes.***
- ***Few bilateral pulmonary nodules for follow up.***
- ***Few splenic hypodense non enhancing focal lesions likely cysts.***
- ***No evidence of hepatic metastatic deposits.***

Dr. All Saber, MSc

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Prince Sultan Cancer Hospital
Radiology Department

Patient Name: Samia Hanna Agaybi A. Madi	Date: 22/09/2024
Exam: MAMMOGRAPHY BOTH	Age: 60 Year(s)
Medical ID: 1383/2024	Pat. ID: 76476

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathologically proven malignant mass on neoadjuvant hormonal therapy for assessment

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

Scattered fibroglandular density (ACR B)

Left breast shows (QO deeply seated spiculated irregular mass lesion of equal density currently still measuring about 1.9x1.8 cm , with clip seen within

No spiculated masses or suspicious microcalcifications on the right breast

Complementary ultrasound revealed left breast irregular spiculated isoechoic mass lesion, seen at 8 o'clock measuring 1.8 x 1.5 cm (was 2 x 1.7 cm in prior study dated)

No right breast solid or cystic masses

Normal skin thickness, and nipple-areola complex of the right breast

Nonspecific right axillary LN

Rather stationary size of pathological left axillary level I and II lymph nodes the largest one measures 1.7 x 3.5 cm compared to 1.3 x 2.9 cm

Free supra-clavicular regions

Opinion:

No radiological response of the left breast (IQ malignant mass and axillary lymph nodes (BIRADS 6)

Immunohistochemistry
[[DOCTYPE:IHC]]